OCD-03 | For therapists | OCD, ages 8 and over

Understanding the model and choosing goals

1 | Purpose, fit and boundaries

Purpose of this unit

Record the therapist's explanation of OCD and treatment options, and formulate a functional goal and shared choice. This is not a protocol or session schedule.

Choice and adaptation

Choose this sheet following professional assessment and with consent. The person may skip questions, answer aloud, or choose another tool. This is proposed original material for review; it provides no score, diagnosis, or promise of an outcome.

Boundaries specific to this unit

ERP means exposure and response prevention within individualized treatment. The sheet does not teach its delivery or promise thoughts will disappear. Do not turn explanations into reassurance rituals or present consent as giving up choice.

Reusing existing material

Unit 2 maps a cycle; unit 3 checks understanding and goals. INT-I04 helps formulate a goal but does not replace explanation and consent for specialized treatment.

A fictional example for discussion

Fictional example: Roni wants to join class activities even when doubt bothers her. The therapist checks understanding and remaining questions without requiring a promise that the feared event will never happen.

> Draft for administrator review. Clinical review and final rights approval are still pending.

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Working together with the worksheet

2 | A conversation about one situation

Opening

Choose a situation the person wants to explore. It need not be the hardest situation. Check that the question is understood and the wording fits the person; their account comes before the therapist's interpretation.

1 | Question for discussion

What did I understand about links between rituals, avoidance and difficulty?

2 | Question for discussion

What options and implications did the therapist explain?

3 | Question for discussion

What personal goal will guide our work together?

4 | Question for discussion

How will we assess functioning and learning without requiring absolute certainty?

5 | Question for discussion

What do I need to make an informed next-step decision?

Closing

Summarize what became clearer and what remains open. Use the existing SES-S01 and SES-S02 for the session framework. There is no need to repeat an assessment or plan already recorded.

Possible difficulties

If the person seeks complete certainty about treatment outcomes, record the question and explore expectations. Resistance or misunderstanding does not mean unwillingness to recover; assess language, pace and fit.

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Adapting for four age groups

3 | Age is a starting point, not an assumption about ability

CH | Children aged 8–11 only

Ages 8–11 only. Read one question and allow a brief or spoken response. Parents add separate observations without deciding the child's thoughts; do not test reporting accuracy. A goal may be a game or activity the child chooses.

Offer one question at a time with little writing. Adult support must not replace the child's own voice.

TE | Adolescents, approximately ages 12 to 17

Ages 12–17. Agree what stays private and what is shared with family or school, subject to safeguarding duties. Embarrassing thought content need not be disclosed. Do not make privacy conditional on treatment consent.

Agree in advance what will be shared and with whom, and explain the privacy limits in this setting. Do not promise absolute confidentiality.

AD | Adults

Connect work to chosen goals, family, employment and existing care. Distinguish rituals, practical needs and value-consistent behavior. Also record preferences and explained alternatives.

OA | Older adults

Preserve adult language, choice and privacy. Assess access, health and desired support without automatically attributing new changes to OCD or assuming reduced ability. Do not assume age limits the person's goals.

Offer reading support, time, and conversation according to preference. Choose help from another person together with the participant, respecting their privacy.

> Each age version needs its own review. Approval of one version does not automatically approve the others.

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Practice, review and next steps

4 | Shared choice and learning from the attempt

Before offering practice sheet P

The P sheet proposes recording decisions and learning within treatment, not ERP instructions. The therapist determines suitability, action, scope and support. Without an agreed plan, use only for discussion. Do not begin exposure or change response prevention based on this sheet.

At the next session

Return to the chosen goal and check understanding, willingness and barriers. Changing a goal or choosing to wait informs planning; it is not failure.

Professional and technical records

Each version has a separate review record covering age suitability, originality, rights, and fictional filling. Reviewer and date fields remain blank until assigned. File checks or a code merge are not clinical approval. Clinical information stays only in the authorized setting, outside the code repository.

Background references

The following links were used for background only. Questions and adaptations were written afresh for the expansion plan; no questionnaire, scale, or protocol was copied. Adult sources do not validate the child versions. This material makes no claim about session count or treatment effectiveness.

Treatment-manual mapping status

Mapping against a full OCD manual and professional approval remain pending. This is original support material for review, not an approved ERP manual or its replacement.

www.nice.org.uk/guidance/cg31/chapter/Recommendations
iocdf.org/about-ocd/ocd-treatment-guide/erp/
kids.iocdf.org/professionals/md/treatment/

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